2.23 Use of Physical Restraints*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Family say he does not know where he is **Objective** • Disoriented • Pulls at lines and tubes	**ACTUAL** Acute confusion related to infection, medication effects and an unfamiliar environment as evidenced by disorientation and pulling at medical devices.	• Kept safe using the least restrictive measures.	• Try alternatives first: reorientation, family presence and close observation. • Apply a restraint only on a physician's order and within policy. • Explain to the family what is being done and why.	• Considered met when he is safe and the restraint is removed as early as possible.
Subjective • Asks for his arms to be freed **Objective** • Wrist restraints in place • Arm movement severely limited	**ACTUAL** Impaired physical mobility related to restraint use as evidenced by limited movement.	• Circulation and joint movement preserved.	• Release the restraint every 2 hours and move the joints through their range. • Check circulation, skin and sensation hourly. • Offer fluids and toileting.	• Goal reached if circulation and skin remain normal.
Subjective *No specific complaint* **Objective** • Restraint applied • Confused	**RISK** Risk for physical injury as evidenced by risk factors: physical restraint and agitation.	• No injury.	• Use the correct size of restraint, tied to the bed frame and never to the rails. • Document the reason, each check and his response. • Keep questioning whether the restraint is still needed.	• Met if he is uninjured.

* **References for this care plan:** [35], [48], [58], [74], [110]. Full entries are listed in the References section of the CPMN book.